

OSPREY ZERO / OFFLINE COLLECTION

First aid.

SAVE BEFORE SERVICE FAILS

A practical collection of complete guides and private planning worksheets. No signup required. This saved edition cannot provide current alerts.

Inside the collection.

Adult hands-only CPR

If an adult suddenly collapses, is unresponsive and is not breathing normally or is only gasping, call emergency services, get an AED and start chest compressions. Push hard and fast in the center of the chest at 100–120 compressions per minute; follow dispatcher instructions.

Using an AED

For an unresponsive person who is not breathing normally, call emergency services and begin CPR. Turn on the AED as soon as it arrives and follow its voice and picture prompts. Do not touch the person during rhythm analysis or a shock.

Severe bleeding

Call emergency services and press firmly directly on the bleeding wound with a dressing or clean cloth. Maintain pressure. For life-threatening bleeding from an arm or leg, use a commercial tourniquet if trained; follow dispatcher instructions.

Adult & child choking

If someone cannot speak, breathe or cough effectively, call emergency services. For a responsive adult or child, give 5 back blows followed by 5 abdominal thrusts, repeating until the blockage clears or they become unresponsive. Use chest thrusts when pregnant or when you cannot encircle the abdomen.

Infant choking

For a baby under one year who cannot cry, cough effectively or breathe, request emergency help and alternate 5 back blows with 5 chest thrusts. Support the head and neck; never give abdominal thrusts to an infant. If unresponsive, begin infant CPR.

Child & infant CPR

If a child or infant is unresponsive and not breathing normally, call the local emergency number, begin CPR and get an AED. Compressions plus breaths are important for children and infants. Follow dispatcher coaching and current hands-on training.

Suspected stroke

Sudden facial droop, arm weakness or speech trouble may be a stroke. Call the local emergency number immediately, note when symptoms began or the person was last well, and stay with them. Symptoms that disappear still need urgent medical evaluation.

Chest pain & suspected heart attack

New chest pressure, squeezing or unexplained discomfort with shortness of breath, sweating, nausea or pain in the jaw, back or arm can be a heart attack. Stop activity, call the local emergency number and rest. Do not drive yourself.

Severe allergy & anaphylaxis

Trouble breathing, throat or tongue swelling, faintness or collapse after an allergen may be anaphylaxis. Use the person's prescribed epinephrine promptly according to its instructions and seek emergency care. Antihistamines do not replace epinephrine.

Burns & scalds

Stop contact with the heat source safely. Cool a thermal burn promptly with clean running water for 5–20 minutes while preventing whole-body chilling, then loosely cover it with a clean nonadherent dressing. Seek urgent help for deep, large, sensitive-area burns or breathing trouble.

Seizures

Keep the person away from injury, time the seizure and do not restrain them or put anything in their mouth. Seek urgent help for a first seizure, one lasting over 5 minutes, repeated seizures without recovery, injury or breathing difficulty.

Suspected opioid overdose

Unresponsiveness with slow, abnormal or absent breathing may be an opioid overdose. Call the local emergency number, give naloxone according to the product instructions if available, and begin CPR with breaths if not breathing normally. Do not delay CPR or an AED for naloxone.

Edition 2026–10–02. First-aid content is source-linked editorial guidance, not independent clinical certification. Seek current training and qualified help when reachable and safe.

FIELD LIBRARY / FULL FIELD GUIDE

Adult hands-only CPR

MEDICAL / EDITION 2026-10-02

QUICK ANSWER / START HERE

If an adult suddenly collapses, is unresponsive and is not breathing normally or is only gasping, call emergency services, get an AED and start chest compressions. Push hard and fast in the center of the chest at 100–120 compressions per minute; follow dispatcher instructions.

Do now.

**Check safely**

01

Ensure the scene is safe and check responsiveness and breathing quickly.

**Call and get an AED**

02

Use speakerphone or send someone to call and bring an AED.

**Give compressions**

03

Place both hands in the center of the chest; compress at least 2 inches, avoiding more than 2.4 inches, and allow full recoil.

**Continue**

04

Follow the AED prompts and dispatcher instructions until responders take over or normal signs of life return.

Keep the plan moving.

IF HELP IS UNAVAILABLE

If a call will not connect, start or continue compressions. Ask another person to keep trying and bring an AED. If alone, use speakerphone while starting CPR rather than waiting for an answer. Do not leave an unresponsive person just to seek better reception.

Watch for.

This adult sudden-collapse guide does not replace infant or child CPR instructions. Gasping is not normal breathing.

People & practical needs.

Give someone the specific task of calling or locating care while another person provides first aid. Keep the patient warm or cool as the condition requires and record what happened and when. Do not put yourself into the same hazard or assume an ambulance has been dispatched until confirmed. For this situation, brief every person on the first action: check safely. Agree who will manage children, pets or mobility needs while that action happens. This adult sudden-collapse guide does not replace infant or child CPR instructions. Gasping is not normal breathing.

Keep essentials within reach.

For adult hands-only cpr, prioritize only items needed for the first action and the fallback: If a call will not connect, start or continue compressions. Ask another person to keep trying and bring an AED. If alone, use speakerphone while starting CPR rather than waiting for an answer. Do not leave an unresponsive person just to seek better reception. Keep the relevant checklist and contact information within reach. Equipment retrieval must not delay the protective step.

Next decisions.

Take a CPR course. Trained rescuers can give breaths according to training; children, infants, drowning and other breathing-related arrests need appropriate CPR with breaths.

First decision: Check safely

Ensure the scene is safe and check responsiveness and breathing quickly. Decide which person can carry this out immediately and what would prevent it in your current location. Do not delay the protective step to complete a household worksheet.

When the original plan fails

If a call will not connect, start or continue compressions. Ask another person to keep trying and bring an AED. If alone, use speakerphone while starting CPR rather than waiting for an answer. Do not leave an unresponsive person just to seek better reception. Reassess the next available location or resource using the warning signs in this guide; do not assume a promised service is operating.

Before changing location

This adult sudden-collapse guide does not replace infant or child CPR instructions. Gasping is not normal breathing. Check the actual route and the needs of the person who will have the most difficulty moving. A destination is not confirmed until access and conditions have been checked.

Questions in the field.

What matters before collecting supplies?

If an adult suddenly collapses, is unresponsive and is not breathing normally or is only gasping, call emergency services, get an AED and start chest compressions. Push hard and fast in the center of the chest at 100–120 compressions per minute; follow dispatcher instructions. Collect items only if doing so does not delay this action.

What should another household member do?

Use a second person to handle communication or dependents while the first carries out the immediate action. For adult hands-only cpr, keep the specific hazard in view: This adult sudden-collapse guide does not replace infant or child CPR instructions. Gasping is not normal breathing.

What should be written down?

Record when the situation changed, what action was taken and what remains uncertain. For this situation, start with: Continue. Follow the AED prompts and dispatcher instructions until responders take over or normal signs of life return. Keep personal notes private; they are not submitted to Osprey Zero.

YOUR SITUATION / PRIVATE WORKSHEET

TIME / LOCATION / CHANGING CONDITIONS

PEOPLE / ACCESS OR MEDICAL NEEDS

NEXT ACTION / PERSON RESPONSIBLE

Sources & limits.

Civilian educational guidance. Linked references may use U.S. emergency systems; local warning methods, laws, services and conditions differ.

Source-linked editorial guidance. Not independent clinical or specialist certification. Training and product-specific instructions remain essential.

Adult hands-only CPR · official guidance

<https://www.redcross.org/take-a-class/cpr/performing-cpr/hands-only-cpr>

Full guide: <https://ospreyzero.com/emergencies/cpr/>

Content updated 2026-10-02. Saved guidance cannot provide live alerts.

FIELD LIBRARY / FULL FIELD GUIDE

Using an AED

MEDICAL / EDITION 2026-10-02

QUICK ANSWER / START HERE

For an unresponsive person who is not breathing normally, call emergency services and begin CPR. Turn on the AED as soon as it arrives and follow its voice and picture prompts. Do not touch the person during rhythm analysis or a shock.

Do now.

**Turn it on**

01

Continue CPR while another helper prepares the AED if possible.

**Expose the chest**

02

Dry a wet chest and place pads according to the AED diagrams.

**Stand clear**

03

Make sure no one touches the person during analysis or shock delivery.

**Resume CPR**

04

Immediately follow the device instructions to continue compressions.

Keep the plan moving.

IF HELP IS UNAVAILABLE

If nobody can bring an AED, continue CPR rather than leaving the person to search alone. If another person is present, ask them to locate the nearest accessible AED while you continue compressions. Follow the device prompts when it arrives.

Watch for.

For a child or infant, use the appropriate pediatric pads or setting when available and follow the device and dispatcher instructions.

People & practical needs.

Give someone the specific task of calling or locating care while another person provides first aid. Keep the patient warm or cool as the condition requires and record what happened and when. Do not put yourself into the same hazard or assume an ambulance has been dispatched until confirmed. For this situation, brief every person on the first action: turn it on. Agree who will manage children, pets or mobility needs while that action happens. For a child or infant, use the appropriate pediatric pads or setting when available and follow the device and dispatcher instructions.

Keep essentials within reach.

For using an aed, prioritize only items needed for the first action and the fallback: If nobody can bring an AED, continue CPR rather than leaving the person to search alone. If another person is present, ask them to locate the nearest accessible AED while you continue compressions. Follow the device prompts when it arrives. Keep the relevant checklist and contact information within reach. Equipment retrieval must not delay the protective step.

Next decisions.

Keep the AED connected until responders take over. Learn the location of nearby devices and obtain training.

First decision: Turn it on

Continue CPR while another helper prepares the AED if possible. Decide which person can carry this out immediately and what would prevent it in your current location. Do not delay the protective step to complete a household worksheet.

When the original plan fails

If nobody can bring an AED, continue CPR rather than leaving the person to search alone. If another person is present, ask them to locate the nearest accessible AED while you continue compressions. Follow the device prompts when it arrives. Reassess the next available location or resource using the warning signs in this guide; do not assume a promised service is operating.

Before changing location

For a child or infant, use the appropriate pediatric pads or setting when available and follow the device and dispatcher instructions. Check the actual route and the needs of the person who will have the most difficulty moving. A destination is not confirmed until access and conditions have been checked.

Questions in the field.

What matters before collecting supplies?

For an unresponsive person who is not breathing normally, call emergency services and begin CPR. Turn on the AED as soon as it arrives and follow its voice and picture prompts. Do not touch the person during rhythm analysis or a shock. Collect items only if doing so does not delay this action.

What should another household member do?

Use a second person to handle communication or dependents while the first carries out the immediate action. For using an aed, keep the specific hazard in view: For a child or infant, use the appropriate pediatric pads or setting when available and follow the device and dispatcher instructions.

What should be written down?

Record when the situation changed, what action was taken and what remains uncertain. For this situation, start with: Resume CPR. Immediately follow the device instructions to continue compressions. Keep personal notes private; they are not submitted to Osprey Zero.

YOUR SITUATION / PRIVATE WORKSHEET

TIME / LOCATION / CHANGING CONDITIONS

PEOPLE / ACCESS OR MEDICAL NEEDS

NEXT ACTION / PERSON RESPONSIBLE

Sources & limits.

Civilian educational guidance. Linked references may use U.S. emergency systems; local warning methods, laws, services and conditions differ.

Source-linked editorial guidance. Not independent clinical or specialist certification. Training and product-specific instructions remain essential.

Using an AED · official guidance

<https://www.redcross.org/take-a-class/aed/using-an-aed>

Full guide: <https://ospreyzero.com/emergencies/aed/>

Content updated 2026-10-02. Saved guidance cannot provide live alerts.

FIELD LIBRARY / FULL FIELD GUIDE

Severe bleeding

MEDICAL / EDITION 2026-10-02

QUICK ANSWER / START HERE

Call emergency services and press firmly directly on the bleeding wound with a dressing or clean cloth. Maintain pressure. For life-threatening bleeding from an arm or leg, use a commercial tourniquet if trained; follow dispatcher instructions.

Do now.

**Protect and call**

01

Check scene safety, use gloves if available and get emergency help.

**Apply direct pressure**

02

Press firmly over the wound and keep pressing until bleeding stops or appropriate help takes over.

**Use trained techniques**

03

Use a tourniquet or wound packing for the appropriate injury if trained, following the device and dispatcher instructions.

**Monitor the person**

04

Keep them from getting cold, check breathing and remain until responders arrive.

Keep the plan moving.

IF HELP IS UNAVAILABLE

Maintain firm pressure and have another person arrange the safest reachable medical care. A tourniquet, if indicated and used, must stay in place until a medical professional takes over; note its application time. Do not keep releasing pressure to check whether bleeding has stopped.

Watch for.

Do not pull an embedded object out; press around it and get help.

People & practical needs.

Give someone the specific task of calling or locating care while another person provides first aid. Keep the patient warm or cool as the condition requires and record what happened and when. Do not put yourself into the same hazard or assume an ambulance has been dispatched until confirmed. For this situation, brief every person on the first action: protect and call. Agree who will manage children, pets or mobility needs while that action happens. Do not pull an embedded object out; press around it and get help.

Keep essentials within reach.

For severe bleeding, prioritize only items needed for the first action and the fallback: Maintain firm pressure and have another person arrange the safest reachable medical care. A tourniquet, if indicated and used, must stay in place until a medical professional takes over; note its application time. Do not keep releasing pressure to check whether bleeding has stopped. Keep the relevant checklist and contact information within reach. Equipment retrieval must not delay the protective step.

Next decisions.

Get urgent medical assessment even if bleeding stops. Take a practical bleeding-control course.

First decision: Protect and call

Check scene safety, use gloves if available and get emergency help. Decide which person can carry this out immediately and what would prevent it in your current location. Do not delay the protective step to complete a household worksheet.

When the original plan fails

Maintain firm pressure and have another person arrange the safest reachable medical care. A tourniquet, if indicated and used, must stay in place until a medical professional takes over; note its application time. Do not keep releasing pressure to check whether bleeding has stopped. Reassess the next available location or resource using the warning signs in this guide; do not assume a promised service is operating.

Before changing location

Do not pull an embedded object out; press around it and get help. Check the actual route and the needs of the person who will have the most difficulty moving. A destination is not confirmed until access and conditions have been checked.

Questions in the field.

What matters before collecting supplies?

Call emergency services and press firmly directly on the bleeding wound with a dressing or clean cloth. Maintain pressure. For life-threatening bleeding from an arm or leg, use a commercial tourniquet if trained; follow dispatcher instructions. Collect items only if doing so does not delay this action.

What should another household member do?

Use a second person to handle communication or dependents while the first carries out the immediate action. For severe bleeding, keep the specific hazard in view: Do not pull an embedded object out; press around it and get help.

What should be written down?

Record when the situation changed, what action was taken and what remains uncertain. For this situation, start with: Monitor the person. Keep them from getting cold, check breathing and remain until responders arrive. Keep personal notes private; they are not submitted to Osprey Zero.

YOUR SITUATION / PRIVATE WORKSHEET

TIME / LOCATION / CHANGING CONDITIONS

PEOPLE / ACCESS OR MEDICAL NEEDS

NEXT ACTION / PERSON RESPONSIBLE

Sources & limits.

Civilian educational guidance. Linked references may use U.S. emergency systems; local warning methods, laws, services and conditions differ.

Source-linked editorial guidance. Not independent clinical or specialist certification. Training and product-specific instructions remain essential.

Severe bleeding · official guidance

<https://www.redcross.org/take-a-class/resources/learn-first-aid/bleeding-life-threatening-external>

Full guide: <https://ospreyzero.com/emergencies/severe-bleeding/>

Content updated 2026-10-02. Saved guidance cannot provide live alerts.

FIELD LIBRARY / FULL FIELD GUIDE

Adult & child choking

MEDICAL / EDITION 2026-10-02

QUICK ANSWER / START HERE

If someone cannot speak, breathe or cough effectively, call emergency services. For a responsive adult or child, give 5 back blows followed by 5 abdominal thrusts, repeating until the blockage clears or they become unresponsive. Use chest thrusts when pregnant or when you cannot encircle the abdomen.

Do now.

**Assess the cough**

01

If the person can cough effectively, encourage coughing and watch closely.

**Call for help**

02

Send someone to call emergency services, or use speakerphone without delaying care.

**Give back blows and thrusts**

03

Lean them forward for 5 back blows between shoulder blades, then give 5 abdominal thrusts; follow dispatcher guidance.

**If unresponsive**

04

Lower them safely and start CPR as directed. Remove only a clearly visible object; never blindly sweep the mouth.

Keep the plan moving.

IF HELP IS UNAVAILABLE

Begin the first-aid sequence immediately while another person calls if possible. If the person becomes unresponsive, lower them safely and begin CPR. Look for a visible object before breaths; never sweep blindly inside the mouth. A delayed ambulance is not a reason to delay first aid.

Watch for.

Do not use abdominal thrusts on an infant under one year; infant care uses back blows and chest thrusts.

People & practical needs.

Give someone the specific task of calling or locating care while another person provides first aid. Keep the patient warm or cool as the condition requires and record what happened and when. Do not put yourself into the same hazard or assume an ambulance has been dispatched until confirmed. For this situation, brief every person on the first action: assess the cough. Agree who will manage children, pets or mobility needs while that action happens. Do not use abdominal thrusts on an infant under one year; infant care uses back blows and chest thrusts.

Keep essentials within reach.

For adult & child choking, prioritize only items needed for the first action and the fallback: Begin the first-aid sequence immediately while another person calls if possible. If the person becomes unresponsive, lower them safely and begin CPR. Look for a visible object before breaths; never sweep blindly inside the mouth. A delayed ambulance is not a reason to delay first aid. Keep the relevant checklist and contact information within reach. Equipment retrieval must not delay the protective step.

Next decisions.

Get medical assessment after thrusts or persistent symptoms. Learn separate infant choking care.

First decision: Assess the cough

If the person can cough effectively, encourage coughing and watch closely. Decide which person can carry this out immediately and what would prevent it in your current location. Do not delay the protective step to complete a household worksheet.

When the original plan fails

Begin the first-aid sequence immediately while another person calls if possible. If the person becomes unresponsive, lower them safely and begin CPR. Look for a visible object before breaths; never sweep blindly inside the mouth. A delayed ambulance is not a reason to delay first aid. Reassess the next available location or resource using the warning signs in this guide; do not assume a promised service is operating.

Before changing location

Do not use abdominal thrusts on an infant under one year; infant care uses back blows and chest thrusts. Check the actual route and the needs of the person who will have the most difficulty moving. A destination is not confirmed until access and conditions have been checked.

Questions in the field.

What matters before collecting supplies?

If someone cannot speak, breathe or cough effectively, call emergency services. For a responsive adult or child, give 5 back blows followed by 5 abdominal thrusts, repeating until the blockage clears or they become unresponsive. Use chest thrusts when pregnant or when you cannot encircle the abdomen. Collect items only if doing so does not delay this action.

What should another household member do?

Use a second person to handle communication or dependents while the first carries out the immediate action. For adult & child choking, keep the specific hazard in view: Do not use abdominal thrusts on an infant under one year; infant care uses back blows and chest thrusts.

What should be written down?

Record when the situation changed, what action was taken and what remains uncertain. For this situation, start with: If unresponsive. Lower them safely and start CPR as directed. Remove only a clearly visible object; never blindly sweep the mouth. Keep personal notes private; they are not submitted to Osprey Zero.

YOUR SITUATION / PRIVATE WORKSHEET

TIME / LOCATION / CHANGING CONDITIONS

PEOPLE / ACCESS OR MEDICAL NEEDS

NEXT ACTION / PERSON RESPONSIBLE

Sources & limits.

Civilian educational guidance. Linked references may use U.S. emergency systems; local warning methods, laws, services and conditions differ.

Source-linked editorial guidance. Not independent clinical or specialist certification. Training and product-specific instructions remain essential.

Adult & child choking · official guidance

<https://www.redcross.org/take-a-class/resources/learn-first-aid/adult-child-choking>

Full guide: <https://ospreyzero.com/emergencies/choking/>

Content updated 2026-10-02. Saved guidance cannot provide live alerts.

FIELD LIBRARY / FULL FIELD GUIDE

Infant choking

MEDICAL / EDITION 2026-10-02

QUICK ANSWER / START HERE

For a baby under one year who cannot cry, cough effectively or breathe, request emergency help and alternate 5 back blows with 5 chest thrusts. Support the head and neck; never give abdominal thrusts to an infant. If unresponsive, begin infant CPR.

Do now.

**Check for an effective cough**

01

A baby who can cough or cry should be watched closely. Do not begin thrusts while the cough remains effective.

**Give 5 back blows**

02

Support the baby face-down on your forearm, braced on your thigh, with the head lower than the chest. Give 5 firm blows between the shoulder blades with the heel of your hand.

**Give 5 chest thrusts**

03

Turn face-up while supporting the head and neck, keeping the head lower. The 2025 AHA update uses the heel of one hand for infant chest thrusts. Alternate with back blows until relieved or unresponsive.

**Change to CPR if unresponsive**

04

Place on a firm flat surface and begin infant CPR. Before breaths, remove an object only if clearly visible. Never sweep the mouth blindly.

Keep the plan moving.

IF HELP IS UNAVAILABLE

Begin the sequence immediately while a helper calls. If alone, use speakerphone if reachable without delay. If the infant becomes unresponsive, change to CPR rather than continuing responsive choking care. A delayed responder is not a reason to pause.

Watch for.

An infant is under one year. AHA's 2025 heel-of-hand update differs from older two-finger choking materials; use current training. Do not hang a baby upside down or shake them.

People & practical needs.

Have a second caregiver locate help and open access while the first provides care. Teach every regular caregiver the infant sequence using hands-on training; older-child abdominal thrusts are not appropriate for babies. Keep small objects and age-inappropriate food out of reach.

Keep essentials within reach.

Keep the action checklist and written contact details accessible. Use the equipment named in this guide only when available and appropriate. Assign someone to check access, batteries and expiry dates before an incident; retrieving a kit must not delay the immediate protective action.

Next decisions.

Seek medical assessment after thrusts, persistent cough, breathing changes or any loss of responsiveness. Review what caused the blockage and safe feeding practices.

Blockage clears

Stop thrusts when effective coughing, crying or breathing returns. Continue observing and obtain assessment after serious symptoms.

An infant is unresponsive

Use Child & infant CPR. Responsive choking thrusts are not the CPR technique.

Questions in the field.

Can I put a finger in the mouth?

Only remove an object that is clearly visible. A blind sweep can push it further in.

YOUR SITUATION / PRIVATE WORKSHEET

INCIDENT / SYMPTOM ONSET / LOCATION

HELP REQUESTED / RESPONSE CONFIRMED

ACTIONS / TIMES / NEXT HANDOVER

Sources & limits.

First-aid summary based on U.S. references. Use the local emergency number; this is not a diagnosis or a substitute for hands-on training.

Source-linked editorial guidance. Not independent clinical or specialist certification. Training and product-specific instructions remain essential.

American Red Cross – Infant Choking

<https://www.redcross.org/take-a-class/resources/learn-first-aid/infant-choking>

AHA – 2025 choking update

<https://newsroom.heart.org/facts/cpr-cardiopulmonary-resuscitation-and-updated-guidelines-2025>

Full guide: <https://ospreyzero.com/emergencies/infant-choking/>

Content updated 2026-10-02. Saved guidance cannot provide live alerts.

Child & infant CPR

MEDICAL / EDITION 2026-10-02

QUICK ANSWER / START HERE

If a child or infant is unresponsive and not breathing normally, call the local emergency number, begin CPR and get an AED. Compressions plus breaths are important for children and infants. Follow dispatcher coaching and current hands-on training.

Do now.

- ☐ **Check quickly**
01 Check responsiveness and normal breathing for no more than 10 seconds. Gasping is not normal breathing. Place on a firm, flat surface.
- ☐ **Call and get an AED**
02 Use speakerphone or send someone. If alone without a reachable phone after an unwitnessed pediatric collapse, give about 2 minutes of CPR before leaving to call and get an AED; follow dispatcher guidance.
- ☐ **Give compressions**
03 For a single rescuer, give 30 compressions at 100–120 per minute, about one-third chest depth: about 2 inches in a child and 1.5 inches in an infant. Use one or two hands for a child; current AHA infant CPR uses one hand or two-thumb encircling hands.
- ☐ **Add 2 breaths and repeat**
04 Open the airway and give 2 breaths, each about 1 second with visible chest rise. Use a neutral head position for infants. If unable or unwilling to give breaths, provide compressions rather than no CPR.
- ☐ **Use the AED**
05 Follow device prompts and pediatric pad instructions. Pads must not touch; use front-and-back placement when needed. Continue until normal signs of life, takeover, exhaustion or an unsafe scene.

Keep the plan moving.

IF HELP IS UNAVAILABLE

Continue CPR while another person seeks help if possible. If breathing returns, watch continuously; be ready to restart if normal breathing stops. Do not stop simply because the expected ambulance time has passed. Rescuer safety and exhaustion can limit what is possible.

Watch for.

This is a single-rescuer summary, not neonatal resuscitation. Two trained rescuers may use a different compression-to-breath ratio. Follow the dispatcher and current course guidance.

People & practical needs.

Plan for two roles: caller/AED finder and rescuer. Rotate compressions with minimal interruption if another capable person is present. Keep the child's age, collapse circumstances and known medical needs ready for handover; new birth resuscitation needs a different protocol.

Keep essentials within reach.

Keep the action checklist and written contact details accessible. Use the equipment named in this guide only when available and appropriate. Assign someone to check access, batteries and expiry dates before an incident; retrieving a kit must not delay the immediate protective action.

Next decisions.

Arrange urgent medical care after resuscitation. Enroll caregivers in current pediatric CPR training before an emergency.

Breathing normally

Do not give compressions to a normally breathing child. Monitor closely and request urgent assessment for unresponsiveness.

No pediatric AED pads

Follow the AED instructions for available pads. Do not delay all resuscitation while looking for specialist equipment.

Questions in the field.

Is adult hands-only CPR enough?

Breaths are especially important in pediatric arrest. Give compressions if breaths are not possible, and seek dispatcher coaching.

YOUR SITUATION / PRIVATE WORKSHEET

INCIDENT / SYMPTOM ONSET / LOCATION

HELP REQUESTED / RESPONSE CONFIRMED

ACTIONS / TIMES / NEXT HANDOVER

Sources & limits.

First-aid summary based on U.S. references. Use the local emergency number; this is not a diagnosis or a substitute for hands-on training.

Source-linked editorial guidance. Not independent clinical or specialist certification. Training and product-specific instructions remain essential.

AHA / AAP – 2025 pediatric basic life support

<https://publications.aap.org/pediatrics/article/157/1/e2025074350/205235/Part-6-Pediatric-Basic-Life-Support-2025-American>

American Red Cross – Child Cardiac Arrest

<https://www.redcross.org/take-a-class/resources/learn-first-aid/child-cardiac-arrest>

American Red Cross – Infant Cardiac Arrest

<https://www.redcross.org/take-a-class/resources/learn-first-aid/infant-cardiac-arrest>

AHA – 2025 CPR guidance

<https://newsroom.heart.org/facts/cpr-cardiopulmonary-resuscitation-and-updated-guidelines-2025>

Full guide: <https://ospreyzero.com/emergencies/child-infant-cpr/>

Content updated 2026-10-02. Saved guidance cannot provide live alerts.

FIELD LIBRARY / FULL FIELD GUIDE

Suspected stroke

MEDICAL / EDITION 2026-10-02

QUICK ANSWER / START HERE

Sudden facial droop, arm weakness or speech trouble may be a stroke. Call the local emergency number immediately, note when symptoms began or the person was last well, and stay with them. Symptoms that disappear still need urgent medical evaluation.

Do now.

- ☐ **Recognize sudden changes**
01 Use FAST: Face droop, Arm weakness, Speech difficulty, Time to act. Sudden loss of balance, vision changes or severe headache can also matter.
- ☐ **Record the time**
02 Write down symptom onset or the last time the person was known well. Do not delay seeking care while completing a checklist.
- ☐ **Protect breathing**
03 Keep them comfortable. If not fully awake but breathing, place on their side when safe. Do not give food, drink or medication to someone with swallowing difficulty.
- ☐ **Watch responsiveness**
04 If unresponsive and not breathing normally, begin CPR and use an AED. Follow dispatcher instructions when available.

Keep the plan moving.

IF HELP IS UNAVAILABLE

Keep attempting to reach urgent medical care while watching breathing and responsiveness. Tell a trusted person the exact location and last-known-well time. Do not make the affected person walk or drive. An inaccessible ambulance service does not make home observation a treatment.

Watch for.

Do not assume FAST rules out stroke; children and some adults have other symptoms. Do not use aspirin as stroke treatment without clinical direction.

People & practical needs.

Speech difficulty can prevent a person from telling others what they need. Use short questions, allow time and avoid asking them to repeat a long history. Keep children with another adult, clear access and prepare the medication list for handover.

Keep essentials within reach.

Keep the action checklist and written contact details accessible. Use the equipment named in this guide only when available and appropriate. Assign someone to check access, batteries and expiry dates before an incident; retrieving a kit must not delay the immediate protective action.

Next decisions.

Pass the symptom timeline to the receiving clinician. Even a brief episode or apparent recovery needs urgent assessment.

If symptoms stop

Keep the request for urgent evaluation active; write when symptoms resolved. Do not cancel care based on improvement alone.

If communication fails

Use a written location and symptom timeline for a trusted relay. Keep the patient accompanied rather than sending them alone to find a signal.

Questions in the field.

Is this heat stroke?

This page concerns a sudden neurological emergency. Heat illness needs immediate cooling; see Extreme heat. Search results cannot distinguish the cause of confusion.

[YOUR SITUATION](#) / [PRIVATE WORKSHEET](#)

INCIDENT / SYMPTOM ONSET / LOCATION

HELP REQUESTED / RESPONSE CONFIRMED

ACTIONS / TIMES / NEXT HANDOVER

Sources & limits.

First-aid summary based on U.S. references. Use the local emergency number; this is not a diagnosis or a substitute for hands-on training.

Source-linked editorial guidance. Not independent clinical or specialist certification. Training and product-specific instructions remain essential.

[American Red Cross – Stroke](#)

<https://www.redcross.org/take-a-class/resources/learn-first-aid/stroke>

[AHA / Red Cross – 2024 first-aid guidelines](#)

<https://cpr.heart.org/en/resuscitation-science/2024-first-aid-guidelines>

Full guide: <https://ospreyzero.com/emergencies/stroke/>

Content updated 2026-10-02. Saved guidance cannot provide live alerts.

FIELD LIBRARY / FULL FIELD GUIDE

Chest pain & suspected heart attack

MEDICAL / EDITION 2026-10-02

QUICK ANSWER / START HERE

New chest pressure, squeezing or unexplained discomfort with shortness of breath, sweating, nausea or pain in the jaw, back or arm can be a heart attack. Stop activity, call the local emergency number and rest. Do not drive yourself.

Do now.

- ☐ **Stop and seek urgent care**
01 Help the person rest in a comfortable position. Request emergency medical assistance and an AED if someone can bring one.
- ☐ **Stay nearby**
02 Monitor breathing and responsiveness. Symptoms can be subtle or come and go; do not wait for a textbook pattern.
- ☐ **Use medicines carefully**
03 Assist with the person's own prescribed medication as directed. Ask a dispatcher or clinician about aspirin; avoid it if contraindicated, allergic, unable to swallow, or uncertain.
- ☐ **If they collapse**
04 If unresponsive and not breathing normally, begin CPR and use an AED promptly. A heart attack can lead to cardiac arrest.

Keep the plan moving.

IF HELP IS UNAVAILABLE

Keep contacting reachable medical care and a trusted local helper while the person rests. Confirm whether assistance is actually coming and ask a clinician about transport when ambulance access fails. The affected person must not drive; household planning cannot replace urgent treatment.

Watch for.

Chest discomfort is not always a heart attack, and absence of chest pain does not exclude one. Do not diagnose it using an online checklist or exertion test.

People & practical needs.

Choose one person to remain with the patient and one to communicate. Prepare a concise medication and allergy list, leave room for first aid and keep dependents supervised. If alone, put the call on speaker and share the location before giving a long history.

Keep essentials within reach.

Keep the action checklist and written contact details accessible. Use the equipment named in this guide only when available and appropriate. Assign someone to check access, batteries and expiry dates before an incident; retrieving a kit must not delay the immediate protective action.

Next decisions.

Arrange urgent evaluation even if pain eases. Record symptom onset and any medicine taken, including dose and time.

Unresponsive but breathing

Continue watching breathing. Use a recovery position if appropriate and safe; do not give oral medication.

No AED nearby

Do not leave CPR to search for distant equipment. Send another person when possible.

Questions in the field.

Can the person wait until morning?

Suspected heart attack needs urgent medical evaluation. Improvement is not proof that the risk has passed.

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American Red Cross – Heart Attack

<https://www.redcross.org/take-a-class/resources/learn-first-aid/heart-attack>

AHA / Red Cross – 2024 first-aid guidelines

<https://cpr.heart.org/en/resuscitation-science/2024-first-aid-guidelines>

Full guide: <https://ospreyzero.com/emergencies/heart-attack/>

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Severe allergy & anaphylaxis

MEDICAL / EDITION 2026-10-02

QUICK ANSWER / START HERE

Trouble breathing, throat or tongue swelling, faintness or collapse after an allergen may be anaphylaxis. Use the person's prescribed epinephrine promptly according to its instructions and seek emergency care. Antihistamines do not replace epinephrine.

Do now.

**Act on serious symptoms**

01

Breathing trouble, swelling or collapse needs immediate care; a rash is not required. Get the prescribed epinephrine and request emergency assistance.

**Follow the actual device**

02

Assist with prescribed epinephrine according to the label, care plan or dispatcher. Different auto-injectors and nasal products have different instructions. Note the time.

**Keep them accompanied**

03

Use a comfortable position; lie down if showing shock when tolerated. Do not make the person stand or walk.

**Monitor and reassess**

04

If symptoms persist, follow the prescribed plan, product instructions or dispatcher for a repeat dose. If unresponsive and not breathing normally, begin CPR and use an AED.

Keep the plan moving.

IF HELP IS UNAVAILABLE

Give prescribed epinephrine without waiting for a callback. Keep requesting urgent care and monitor breathing. Record the dose time and keep the used device for handover. Do not treat improvement as proof that the reaction has ended.

Watch for.

Do not substitute an antihistamine, food or drink for urgent treatment. Avoid a universal device hold-time: follow the product label.

People & practical needs.

Ask caregivers where the prescribed medicine is kept before an outing. Practice with the correct trainer device, explain who will fetch it and who will stay with the person, and check storage and expiry instructions. A school or childcare plan should travel with the child.

Keep essentials within reach.

Keep the action checklist and written contact details accessible. Use the equipment named in this guide only when available and appropriate. Assign someone to check access, batteries and expiry dates before an incident; retrieving a kit must not delay the immediate protective action.

Next decisions.

Urgent medical assessment is needed after epinephrine, including when symptoms improve. Replace used supplies and review the allergy action plan with a clinician.

No epinephrine available

Request urgent medical assistance immediately, stay with the person and monitor breathing. Do not improvise an injection or medication dose.

A second device is available

Check the person's action plan and current device instructions. Record each administration separately for the clinician.

Questions in the field.

Can anaphylaxis happen without hives?

Yes. Serious breathing or circulation symptoms after a possible allergen need urgent action even without visible skin changes.

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[American Red Cross – Allergic Reaction Anaphylaxis](#)

<https://www.redcross.org/take-a-class/resources/learn-first-aid/allergic-reaction-anaphylaxis>

[AHA / Red Cross – 2024 first-aid guidelines](#)

<https://cpr.heart.org/en/resuscitation-science/2024-first-aid-guidelines>

Full guide: <https://ospreyzero.com/emergencies/anaphylaxis/>

Content updated 2026-10-02. Saved guidance cannot provide live alerts.

FIELD LIBRARY / FULL FIELD GUIDE

Burns & scalds

MEDICAL / EDITION 2026-10-02

QUICK ANSWER / START HERE

Stop contact with the heat source safely. Cool a thermal burn promptly with clean running water for 5–20 minutes while preventing whole-body chilling, then loosely cover it with a clean nonadherent dressing. Seek urgent help for deep, large, sensitive-area burns or breathing trouble.

Do now.

- ☐ **Stop the exposure**
01 Move away from the heat safely. Do not touch an electrically injured person until the power source is safely isolated.
- ☐ **Cool the burned area**
02 Use clean running water promptly for 5–20 minutes. Keep the rest of the person warm, especially children; stop whole-body chilling.
- ☐ **Remove tight items**
03 Remove jewelry and tight items before swelling if they are not stuck. Do not pull away material adhering to the skin.
- ☐ **Cover and seek assessment**
04 After cooling, loosely cover with a clean nonadherent dressing. Get prompt care for deep burns, burns larger than the person's palm or those involving face, hands, feet or genitals.

Keep the plan moving.

IF HELP IS UNAVAILABLE

Use clean available water to cool the thermal burn while seeking assessment; keep the unburned body warm. Breathing difficulty or smoke around the mouth or nose requires urgent help. Do not turn lack of specialist access into a reason to apply household remedies.

Watch for.

Do not apply ice directly, butter or toothpaste. Do not break blisters. This cooling sequence is for thermal burns, not every chemical exposure.

People & practical needs.

A child can cool too quickly during whole-body showers. One caregiver should watch temperature and breathing while another manages water and contact with care. Keep labels or exposure details when chemicals or electricity were involved, without carrying contaminated items into a clean area.

Keep essentials within reach.

Keep the action checklist and written contact details accessible. Use the equipment named in this guide only when available and appropriate. Assign someone to check access, batteries and expiry dates before an incident; retrieving a kit must not delay the immediate protective action.

Next decisions.

Seek professional assessment for serious burns and worsening pain, redness or fever. Chemical and electrical injuries need their own assessment even when skin damage looks small.

No running water

Use clean available water and seek clinical guidance; do not use floodwater or contaminated water. Avoid improvised creams while awaiting assessment.

Smoke inhalation

Breathing trouble, facial burns or soot around nose or mouth needs urgent medical help, even if the skin burn seems limited.

Questions in the field.

Should I wrap it tightly?

No. Use a loose clean nonadherent covering after cooling; swelling can make a tight wrap dangerous.

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American Red Cross – Burns

<https://www.redcross.org/take-a-class/resources/learn-first-aid/burns>

AHA / Red Cross – 2024 first-aid guidelines

<https://cpr.heart.org/en/resuscitation-science/2024-first-aid-guidelines>

Full guide: <https://ospreyzero.com/emergencies/burns/>

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FIELD LIBRARY / FULL FIELD GUIDE

Seizures

MEDICAL / EDITION 2026-10-02

QUICK ANSWER / START HERE

Keep the person away from injury, time the seizure and do not restrain them or put anything in their mouth. Seek urgent help for a first seizure, one lasting over 5 minutes, repeated seizures without recovery, injury or breathing difficulty.

Do now.

**Make space**

01

Move dangerous objects away. Protect the head gently if possible, without holding the person down.

**Time and observe**

02

Note the start and stop times and whether there are repeated seizures. Ask someone to contact help when urgent criteria apply.

**Protect breathing afterward**

03

When safe, position on their side if breathing but not fully awake. Stay with them and monitor responsiveness.

**Act if normal breathing stops**

04

If unresponsive and not breathing normally, begin CPR and use an AED. Do not assume abnormal breathing is simply a normal recovery.

Keep the plan moving.

IF HELP IS UNAVAILABLE

Continue protecting from hazards and observing breathing while a helper seeks medical access. Use prescribed rescue medicine only under the existing care plan and appropriate training. If urgent criteria apply, keep seeking care rather than leaving the person alone to recover.

Watch for.

Never force something into the mouth, give food or drink before fully alert, or restrain movements.

People & practical needs.

Reduce noise and spectators and give the person privacy. Afterward, speak calmly and explain what happened. Assign dependents to another adult and prepare the care plan, medication list and seizure timeline for a trusted receiving clinician.

Keep essentials within reach.

Keep the action checklist and written contact details accessible. Use the equipment named in this guide only when available and appropriate. Assign someone to check access, batteries and expiry dates before an incident; retrieving a kit must not delay the immediate protective action.

Next decisions.

Seek urgent help for seizures in water, during pregnancy, in a baby under 6 months or without return to baseline within 5–10 minutes. A known condition does not remove new warning signs.

An established epilepsy plan exists

Follow the person's prescribed plan and note medicine times. New or unusually prolonged symptoms still need assessment.

In a hazardous location

Move only as needed to escape immediate danger. Do not create a second casualty by entering water, traffic or electrical hazards.

Questions in the field.

Can they swallow their tongue?

No. Putting an object in the mouth can cause choking, dental injury or a bite injury to the helper.

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[American Red Cross – Seizures](#)

<https://www.redcross.org/take-a-class/resources/learn-first-aid/seizures>

[AHA / Red Cross – 2024 first-aid guidelines](#)

<https://cpr.heart.org/en/resuscitation-science/2024-first-aid-guidelines>

Full guide: <https://ospreyzero.com/emergencies/seizures/>

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FIELD LIBRARY / FULL FIELD GUIDE

Suspected opioid overdose

MEDICAL / EDITION 2026-10-02

QUICK ANSWER / START HERE

Unresponsiveness with slow, abnormal or absent breathing may be an opioid overdose. Call the local emergency number, give naloxone according to the product instructions if available, and begin CPR with breaths if not breathing normally. Do not delay CPR or an AED for naloxone.

Do now.

**Check scene and breathing**

01

Avoid needles, powders and traffic hazards. Check responsiveness and normal breathing quickly; gasping or gurgling is not reassuring.

**Request help and equipment**

02

Send someone to call and bring naloxone and an AED when available. Share the exact location and breathing status.

**Support breathing and circulation**

03

If unresponsive and not breathing normally, start CPR with breaths if able and trained; use an AED promptly. Give compressions if breaths are not possible.

**Use naloxone and observe**

04

Follow the actual device instructions, including repeat dosing. Do not interrupt CPR to wait for a response. If normal breathing returns, place on their side as appropriate and keep monitoring.

Keep the plan moving.

IF HELP IS UNAVAILABLE

Give available naloxone and resuscitation promptly while a helper seeks care. Keep monitoring after improvement: opioids can outlast naloxone and breathing can worsen again. Stay until a capable person takes over or the scene becomes unsafe.

Watch for.

Do not let the person "sleep it off," put them in a cold shower or give food or drink. Naloxone is not a substitute for breathing support or urgent care.

People & practical needs.

Keep naloxone accessible wherever overdose risk exists and learn how the supplied product works. Share emergency roles without stigma; the immediate objective is breathing and medical care. Keep children and bystanders away from needles and unverified substances.

Keep essentials within reach.

Keep the action checklist and written contact details accessible. Use the equipment named in this guide only when available and appropriate. Assign someone to check access, batteries and expiry dates before an incident; retrieving a kit must not delay the immediate protective action.

Next decisions.

Medical evaluation is still needed after naloxone. Note dose times and changes in breathing and give this information during handover.

They begin breathing normally

Continue monitoring, use a recovery position when appropriate and be prepared to restart CPR if normal breathing stops.

No naloxone available

Continue resuscitation and seek urgent care. Do not delay CPR while searching distant locations for medicine.

Questions in the field.

Does naloxone replace CPR?

No. CPR and AED use remain essential when the person is unresponsive and not breathing normally.

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[American Red Cross – Opioid Overdose](#)

<https://www.redcross.org/take-a-class/resources/learn-first-aid/opioid-overdose>

[AHA / Red Cross – 2024 first-aid guidelines](#)

<https://cpr.heart.org/en/resuscitation-science/2024-first-aid-guidelines>

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